TRUE NORTH SYSTEMS

# Internal Business Development Brief

True North Continuity — Healthcare Record Archive

INTERNAL · CONFIDENTIAL

## WHAT WE'RE SELLING

A distributed, intrusion-safe healthcare record archive built on a private relay network. We continuously capture ADT events and archive them across geographically separated nodes using Shamir's Secret Sharing for encryption key distribution. Two core value propositions:

1. **Ransomware resilience.** If the primary EMR goes down, our archive reconstructs a functional secondary EMR from the ADT event stream — a live, moving census of every patient.
2. **Exfiltration protection.** If an attacker breaches an archive node, they get ciphertext and one key share. Neither is sufficient to reconstruct any patient record. No existing backup vendor offers this.

## MESSAGING PILLARS

Use these consistently across BD conversations. Tune emphasis to the buyer's role.

PILLAR 1

### "Recovery is solved; exfiltration isn't."

Competitive wedge. Every incumbent protects against deletion. None protect against theft.

PILLAR 2

### "Secondary system = can shut down to protect."

Business model advantage. We aren't the primary EMR. We can take protective actions that primary systems cannot.

PILLAR 3

### "Visible trust beats compliance theater."

Sales conversion. The live verification display makes the system self-evident to the buyer.

PILLAR 4

### "Portability without pain."

Secondary conversation opener for prospects not ready to discuss ransomware.

## COMPETITIVE LANDSCAPE

| | RUBRIK / COHESITY / VEEAM | EMRRESCUE | EPIC CRO | HIES | TRUE NORTH |
|---|---|---|---|---|---|
| Immutable backups | Yes | No | Yes | No | Yes |
| Clinical continuity | No | Yes (mini-EHR) | Yes (read-only) | Partial | Yes (secondary EMR) |
| Exfiltration protection | **No** | **No** | **No** | **No** | **Yes (SSS)** |
| CIO live verification | No | No | No | No | **Yes** |
| AD-independent auth | Varies | Cloud | Cloud | Varies | **Yes (own mTLS)** |

The exfiltration column is the gap we fill. Everything else is table stakes we deliver alongside.

## THE LIVE PROSPECT

We have a warm, friendly contact — a CMIO at a major health system: 30 hospitals, 200+ sites, $6B+ revenue, six states. In November 2023, this system suffered a catastrophic ransomware attack that took their entire network offline, including Epic. Ambulances were diverted. Procedures were cancelled. Recovery took weeks. He lived through the exact scenario we prevent. A second contact — a CIO at a single hospital within a wider hospital system — experienced a similar attack in early 2025. These are just our immediate, relatively small circles.

> *"If you'd had our network last November, your doctors wouldn't have noticed the Epic outage."*

## MULTI-STAKEHOLDER SALE

Map pillars to the buyer's role:

| STAKEHOLDER | LEAD WITH | SECONDARY |
| --- | --- | --- |
| CIO | Exfiltration gap, backup-as-strength | Data portability, cost reduction |
| CMIO | Clinical continuity, live verification | Reduced regulatory exposure |
| CISO | AD bypass, SSS key-splitting | HIPAA Safe Harbor alignment |
| Compliance / Legal | Recognized security practices, audit evidence | Verification logs as compliance artifacts |

## HANDLING OBJECTIONS

**"This is just another backup solution."**

No. Backup protects against data loss. We protect against data theft. If someone steals data from a Rubrik appliance, it's compromised. If they steal from one of our nodes, they get a key share and ciphertext — insufficient to reconstruct any record.

**"What if YOUR system goes down?"**

Nothing happens. We're secondary. The primary EMR continues. We can shut ourselves down aggressively on intrusion suspicion — zero clinical impact. We shut down to protect records; Epic shuts down to protect its own infrastructure.

**"SSS isn't built yet."**

Correct. The architecture is validated, the math is proven (Shamir, 1979 — used in Bitcoin custody, academic healthcare papers), and the key-splitting model is straightforward. We need a funded pilot to build production. That's what we're raising for.

**"Are you claiming HIPAA safe harbor automatically?"**

No. We align with recognized security practices that support reduced exposure. We do not claim automatic exemption from breach notification.

**"Why not just use Epic CRO or EMRrescue?"**

They restore access. They do not prevent exfiltration. Stolen data is still stolen. We add the missing security layer.

### THE "NOBODY TALKS ABOUT IT" CHALLENGE

Hospitals avoid buying "anti-ransomware" because it admits vulnerability. Navigate by:

- **Reframe** as operational resilience and data availability, not breach defense
- **Compliance-driven entry:** HIPAA Safe Harbor incentivizes recognized security practices — we help them demonstrate compliance
- **The Doubting Thomas pitch:** Give them a tool that makes their data visible and verifiable. The pitch is empowerment, not fear.
- **Insurance as channel:** If insurers see our architecture reducing breach impact, they mandate it — creating pull without the hospital admitting vulnerability
- **Peer networks:** A live demo at HIMSS or CHIME propagates through CIO/CMIO circles

### DATA PORTABILITY (SECONDARY OPENER)

For prospects not ready to discuss ransomware, lead with: "High compliance doesn't have to mean hard and slow." Adding or removing nodes is trivial. Data replicates via policy. We can stand up a dedicated network on customer infrastructure without their IT team understanding the internals. This addresses daily operational pain — vendor lock-in, migration friction, costly managed cloud storage — that every CIO deals with.

### SECURITY CLAIMS GUARDRAILS

Be precise in all BD conversations:

**SAY**

"Key-splitting across independent zones prevents a single breach from yielding patient records."

**DON'T SAY**

"HIPAA exemption" or "zero-knowledge by default."

**SAY**

"Supports recognized security practices under HIPAA Safe Harbor."

**DON'T SAY**

"Eliminates reporting requirements."

**SAY**

"The encryption key is distributed so no single node holds it."

**DON'T SAY**

"The data is mathematically useless." (The ciphertext is on the node; it's the key that's split.)

## BEYOND HEALTHCARE (FUTURE VERTICALS)

- **Insurance companies** — more valuable as a distribution channel (mandating our solution for hospitals) than as direct customers initially
- **Legal** — defensible audit trails, but small market with entrenched e-discovery vendors (Relativity, Mimecast)
- **Finance** — deeply entrenched incumbents (IBM, Veritas), heavy procurement. Not near-term.
- **Government** — interesting long-term, but FedRAMP certification and 12-24 month procurement cycles